

Jacquín
Hypnosis Academy
Since 1999

THE
ARROW
TECHNIQUE

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The Arrow Technique Workbook

This is the workbook for The Arrow Technique, developed by Freddy Jacquin, the founder of the Jacquin Hypnosis Academy, run by Freddy and his son Anthony Jacquin. It should be studied in conjunction with The Arrow Technique video.

Three full transcripts, for three different applications of The Arrow Technique are included.

One of these transcripts is broken down and analysed in detail, to help illustrate each of the principles and techniques that make up The Arrow Technique.

The booklet includes some adapted notes drawn from the book 'Reality is Plastic: The Art of Impromptu Hypnosis' by Anthony Jacquin. These are applicable to all demonstrations of hypnosis.

You will also find notes and transcripts of two additional techniques mentioned in The Arrow Technique training video. These are The Jacquin Fingerlock and The Jacquin Power Lift Induction. Although it is not necessary to use these additional techniques to make use of The Arrow, and it is not necessary to deliver the technique in a hypnotic context, Freddy and Anthony often do.

Please note, this video and booklet are not intended to act as a substitute for full hypnosis or therapy training. If you plan to deliver The Arrow Technique in a hypnotic context, it is up to you to ensure you are appropriately qualified and meet any local or national laws regarding the use of hypnosis.

Use the booklet and the transcripts included to help review the concepts and deepen your understanding of this wonderful tool.

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Introduction to The Arrow Technique

Freddy Jacquin originally developed The Arrow Technique as a means to provide rapid relief to those suffering with chronic pain conditions.

The Arrow Technique has since proven to be a wonderful utility tool for relieving suffering of many kinds. It can be used to help reduce or eliminate physical pain, and also for provoking change in those with a variety of emotional and psychological conditions.

Chronic pain remains a significant and frustrating burden for both individuals and society. Standard medical treatment for chronic pain is often inadequate, and it is common for patients to seek costly treatments from multiple healthcare professionals, often without significant relief. Growing awareness of the limitations of currently available pain treatments make the use of suggestion-based techniques and self-hypnosis an attractive component of pain treatment.

The empirical support for hypnosis for chronic pain management has flourished over the past two decades. Clinical trials show that hypnosis is effective for reducing chronic pain, although outcomes vary between individuals. The findings from these clinical trials also show that hypnotic treatments have a number of positive effects beyond pain control. Despite this many hypnotherapists, as well as practitioners of other therapeutic approaches, do not have a clear approach to dealing with stubborn pain conditions.

This course will give you a clear understanding of one way to provide significant relief for chronic pain conditions. Once that is understood you will learn how to apply this approach to a variety of other issues.

The nature of the techniques shared, means that being a complete beginner does not put you at any great disadvantage; the technique is easy to learn and apply. It is not necessary to be a hypnotherapist to use this technique, nor is it necessary to deliver the technique in a hypnotic context. All that is required is that you deliver it with conviction.

You have the potential to eliminate a vast amount of unnecessary pain and suffering.
Enjoy.

Hypnosis: No Introduction Necessary

Whether someone believes in hypnosis or not, whether they have experienced it or not, they are still likely to have a strong personal concept of what hypnosis is and how it typically proceeds. This may be based on something they have read, seen in a movie or in a live show. They may have first-hand experience as a subject or spectator or just be relying on ideas from fiction, hearsay or urban myth. Even children under ten often have a concept of hypnosis.

So before we begin it might be useful to ask yourself a few questions and note what answers you come up with.

What do you think hypnosis is? What images spring to mind when you think of hypnosis, hypnotists and the hypnotised? Which words or phrases would a hypnotist most commonly use? What does it feel like to be hypnotised? How do people act when hypnotised? Do you think you can be hypnotised? Can hypnotists make you do things against your will? What would miraculous pain control look like?

When you have learnt more about hypnosis and accept yourself as The Hypnotist, it is likely that your ideas about what hypnosis is and how it can be used will be quite different from the view of the general public. It is wise not to forget what most people believe it to be, what they imagine the experience will be like and what phenomenon they think they are witnessing. Using your subjects' mental models of hypnosis is fine, even if they are not that accurate, just as long as their model does not leave them afraid of hypnosis for some reason. If it does then it is useful to remove their fears prior to hypnotising them, and this is simple enough to do. If their fear is that you will be able to hypnotise them that is fine, as long as you can get them to engage.

I have asked hundreds of students of hypnosis, thousands of therapy clients and many hypnotists the question, 'What do you think hypnosis is?'

The answers vary quite dramatically, especially amongst hypnotists. However several common themes run throughout the public perception of this art.

Sleep, trance, a relaxed state, mind control, swinging watches, stage hypnosis, getting in touch with the subconscious, telling someone what to do, a comfy chair, a slow sleepy or gravelly voice, a snap of the fingers, spirals, being under a spell and being out of control. These are all common ideas when people try to give you a sense of what they think hypnosis is all about.

Images of people slumped in a chair with their eyes closed, under the mesmerising gaze and command of The Hypnotist might come to mind. A row of empty chairs, sleeping subjects, people doing ridiculous things or experiencing amnesia for what they have done are also relatively common ideas.

Even in our modern, rather sceptical society, popular belief often still ascribes some significance to the devices and rituals of hypnosis, such as making passes with the hands, spinning hypnotic watches, spirals, the hypnotic stare and the authoritarian command. Several years ago I received a pocket watch on a chain as a present. The first time I got it out to show some friends in a pub, they scattered, turning away—as if looking at it would immediately put them under. It is an iconic object that seems sewn into the history of hypnosis itself and it is to history that we may turn when trying to understand more about a subject.

In his incredibly detailed written study of Primitive Mythology, looking back over 40,000 years of human activity, Joseph Campbell sets out the task ahead of him for the benefit of the reader. He quotes Thomas Mann from the opening of his mythologically conceived tetralogy, *Joseph and His Brothers*, ‘Very deep is the well of the past. Should we not call it bottomless?... No matter to what hazardous lengths we let out our line they still withdraw, again and further into the depths.’ (2)

To get to the roots of hypnotism prior to the mid-eighteenth century, we face a similar proposition. For once you start to follow the threads of historical influence, the roots seem to run as deep as the birth of civilization itself, right through the development of art, science, healthcare and psychology. This can be seen when we peer into very thin cracks of light in the darkness of time gone by and not summarily and sketchily as a mere footnote to the main subject. It is at this point that we must stop just being historiographers and instead adopt the view of the interested psychologist, sociologist, anthropologist, artist or philosopher. This allows a broader understanding beyond ‘what has happened’ and some greater insight into the mind, the role hypnotism has played in the development of science and psychology, the role it plays in society today and the potential it has to provide a window on some of the most fascinating and entrenched mysteries of what it is to be human.

Documents from the eighteenth century help us understand much about the purported foundations of hypnotism in the Western world, laid by Franz Anton Mesmer (1734—1815). Many historical accounts begin there. However the practice of hypnosis in the broad sense that we understand today did not emerge until the nineteenth century with the work of James Braid (1795—1860). The theory and approach of Mesmer was very different from that of Braid. So if we are going to retrospectively shoehorn the ‘Animal Magnetism’ of Mesmer into a history of hypnotism, why stop there? If you really want to get into the history of this subject then we should look further back and identify from where his ideas originated. When we do this it seems there is much in the modern, ancient and even the prehistoric record of man that might represent hypnotism. This may involve broadening our definition of hypnotism, widening our ‘hypnoscope’, to include not just the traditional practice of hypnosis, but also other practices that provoked the common phenomena of hypnosis as we understand them today. This allows us to assess any parallels in the historical record that we can learn from. It is clear to me that there are many such parallels. In my opinion there is a thin line of descent that leads from The Hypnotist all the way back to The Shaman.

It is beyond the scope and remit of this book to provide such a comprehensive history. Fortunately there are already several excellent books that you can obtain that do. These will make clear to you on whose shoulders you are standing when you learn and use hypnosis. We will address just a few of the key figures.

Although methods of inducing trance may have been used for the purposes of the Shaman for healing, dealing with crisis and as an aid to creativity for thousands of years, most histories of hypnosis trace the trance we equate with hypnosis back to the work of an Austrian physician, Franz Anton Mesmer. Mesmer came up with a theory and a way of treating people that by all accounts helped many to health. His ideas were based almost entirely on poorly-tested hypothesis and faulty science, but they led to more accurate ideas about hypnosis in the nineteenth century.

Mesmer believed that among all the fields known to science at that time there was another field, which might be called an animate field or fluid or life force. He defined good health as the free flow of this field or fluid through thousands of channels in our bodies. Illness resulted from obstructions to the free flow of this fluid. Overcoming these obstacles and restoring flow restored health. When nature failed to do this spontaneously, contact with a conductor of ‘animal magnetism’ was a necessary and often a sufficient remedy. Mesmer, in other words, believed that he was a conductor of animal magnetism and that this could influence the flow of the fluid-like life force. Mesmer aimed to aid Nature’s effort to heal. He treated patients

both individually and in groups. With individuals he would sit in front of his patient with his knees touching the patient's knees, pressing the patient's thumbs in his hands, looking fixedly into the patient's eyes. Mesmer also made 'passes', moving his hands from the patient's shoulders down along their arms.

Prior to this it was common practice to do this with magnets. Many patients felt peculiar sensations or had convulsions that were regarded as crises that were supposed to bring about the cure. (3)

In the nineteenth century the idea that there was some invisible fluid, energy or influence travelling from The Hypnotist to the subject, crumbled under scrutiny. Hypnosis instead began to be described in physiological and psychological terms as some kind of special or unique state.

Several influential definitions describe hypnosis as a 'state' of some kind. Just what kind of state however is not as clear. A 'state like sleep', 'a unique or special state of mind', 'a trance state', 'a state of fascination', 'a state of focus', 'any altered state' and of course, 'a deeply relaxed state.'

State-based definitions have some value but upon examination can all be found to be equally unsatisfactory.

We are always in a state of some kind, arguably never in the same state twice. Many studies have found little difference between someone in hypnosis and someone in a normal state. Therefore critics of the state-based view suggest that hypnosis cannot be defined in terms of state. Some go as far as to say that because hypnosis cannot be proved to be a unique state, hypnosis does not exist. Brain imaging can identify physiological markers that make clear a distinction between someone who is responding to a specific suggestion and someone who is simply acting. However no consistent and reliable evidence exists for an independent hypnotic or trance state.

It has been argued that in trying to understand hypnosis, state versus non-state theories only offer us a false dichotomy. We might do better to accept the lack of evidence for a unique state, and instead focus on special

process, social-psychological or cognitive-behavioural models.

These more modern views of hypnosis can describe its subjective nature as points on a continuum, rather than as a dichotomy. (4)

Another way to define hypnosis is as an art. Certainly it is possible to be an excellent hypnotist without knowledge of psychology or brain function or state theory. Talent, flair and the twin forces of personality and enthusiasm will take you far as a hypnotist. So will accepting that hypnosis is really just the artful application of suggestion to someone who is focussed on the ideas The Hypnotist is presenting.

Hippolyte Bernheim (1840—1919) the father of twentieth-century hypnosis, famously said:

‘It is suggestion that rules hypnotism’

Hippolyte Bernheim. (5)

He believed hypnosis was inherently a suggestion-based process. It is a useful assumption, although as difficult to get to grips with scientifically as the state known as trance.

Eventually hypnosis became viewed by many as something for which the subject is responsible, or more accurately, capable of, given the right instruction. This eventually led some to conclude that all hypnosis is in fact self-hypnosis.

In the early twentieth century the social-psychological theory, expressed in the work of Theodore R. Sarbin (1911—2005) and Theodore X. Barber (1927—2005), became increasingly influential. Hypnosis is described in this theory as a peculiarity of the social relationship between The Hypnotist and the subject—The Hypnotist and the subject ‘playing’ their parts as they believe they should. In other words social compliance or role-play is considered to be a key factor. (6, 7)

These ideas were developed further in the late twentieth century into the socio-psychological and cognitive-behavioural views of hypnosis, expressed by Nicholas Spanos (1942—1994) and Irving Kirsch (born 1943) most notably.

Spanos argued that many of the actions performed under hypnosis can be explained with reference to social-psychological and cognitive hypotheses. He alleged that the experience and behaviours associated with hypnosis are acted out by the person being hypnotised in

accordance with cues gleaned from the social context, expectations and setting, although the subject may experience their behaviour with a sense of involuntariness. (8)

This sense of involuntariness, also known as the classic suggestion effect, might well be the key to qualifying an experience as hypnotic.

Kirsch demonstrated that what people experience when hypnotised, including the classic suggestion effect, depends largely on what they expect to experience. His theory is supported by research showing that both subjective and physiological responses can be altered by changing people's expectations.

When the effects of a medicine depend on its psychological meaning, rather than the specific ingredients that it contains it is called a placebo. Placebo effects are well documented and point to a basic principle of human experience and behaviour: When people expect changes in their own responses and reactions, their expectations can produce those changes.

These self-fulfilling response expectancies can be a cause of psychological problems or an essential part of psychological treatment. (9)

According to Kirsch response expectancy is the common ingredient behind both placebo treatments and hypnosis. Like a placebo, hypnosis produces therapeutic effects by changing patients' expectancies. One legitimate barrier to the use of placebo by clinicians is that their use entails deception. The placebo is presented as a genuine pharmacological treatment when it is not. By contrast, hypnosis is presented honestly as a psychological procedure. For this reason Kirsch has characterised clinical hypnosis as a 'non-deceptive placebo.' (10, 11)

It can be argued that all hypnotic experiences take place in the realm of imagination. There isn't really a balloon lifting up the subject's hand or glue holding the subject's hands together. The hypnotist often begins by providing instructions to the subject to imagine something such as a mental image. There is often a point where the act of imagining something and the response to that becomes automatic or seemingly involuntary. It is this subjective feeling of involuntariness, not the vividness of what is imagined, that gives hypnotic experiences their hallucinatory quality. (12, 13)

The debate about the precise nature of hypnosis has gone on for decades and will probably continue to do so. For our purposes we will look at the definitions of arguably the most

influential hypnotists that have walked the planet, James Braid, Milton Erickson and Dave Elman.

James Braid caused a paradigm shift from the mesmerists of the eighteenth and early nineteenth century. He not only popularised the terminology we still use, but at a later date rejected it as misleading. Braid was a doctor and after observing a demonstration of mesmerism believed he had figured out why people went into this peculiar state, and it had nothing to do with an invisible magnetic fluid. He suggested a physical basis for hypnosis. His initial insightful but inaccurate view was that the mesmerised state was caused by the tiring of an optic nerve as the eye fixated—hence the association with focussing on spinning watches, or in his case a silver cigarette case. It seems he initially missed the fact that his verbal suggestions to his subjects that their eyes would feel tired were also having an effect.

Braid was essentially an open-minded rationalist, inspired with the truest scientific spirit. He eagerly seized upon all fresh facts and altered his theories in accordance with them. Later in his career he shifted emphasis from a physical description to one concerning the subjective nature of hypnosis and the effect of suggestion. Braid noted that it is not just the gaze that becomes fixated, but the mind's eye as well. In other words, when hypnotised the mind becomes locked around a single idea. (14)

'The real origin and essence of the hypnotic condition is the induction of a habit of abstraction or mental concentration, in which, as in reverie or spontaneous abstraction, the powers of the mind are so much engrossed with a single idea or train of thought, as... to render the individual unconscious of, or indifferently conscious to, all other ideas, impressions, or trains of thought.'

James Braid. (15)

Note that Braid says hypnosis renders the individual unconscious or indifferently conscious to all other ideas. This is important. When hypnotised you can still have an experience that you can reflect on as you are having it and, as far as you are concerned, a fully conscious experience. For example if you are hypnotised to believe you cannot remove your hand from your face because it has been glued there, you are still able to reflect on the fact that it is stuck and even wonder why it is stuck. However the only reality you have is that it is stuck nonetheless. If you are hypnotised to believe that beer bottle tops are coins then even when it is pointed out that they are bottle tops you will know without doubt or question that they are not bottle tops, they are coins, and will accept them as such. You are indifferent to ideas

other than the one your mind has locked onto as reality. The hypnotist directs the subject's perception of reality by locking the mind around ideas.

Braid went further still in his later works and positioned suggestion, not as being explanatory of hypnotic phenomena, but as the artifice used to excite them.

From the time of Braid right up into the mid-twentieth century hypnosis was typically induced using a direct and authoritarian approach, encouraging the concentration of attention.

In the twentieth century Milton H. Erickson (1901—1980) caused a seismic shift in the way to induce hypnosis, developing a permissive and indirect approach that is very popular with twenty-first century hypnotherapists. By the end of his career he appeared to simply be having conversations with his patients who would go into hypnosis without any mention of the word hypnosis. Of course Erickson knew exactly what he was saying, what he was doing and why it caused hypnosis. His insights with regard to personal change have had a profound impact on modern therapy.

I encourage you to read his work and you will discover many incredible ways you can use hypnosis. He experimented with hypnosis pretty much every day from 1920 to 1980. He covered a lot of ground. Because his permissive approach to hypnosis is so popular, it is often overlooked that Erickson was a master of rapid, direct, impromptu hypnosis too and would use it just as readily as the more cultivated covert or indirect approach. It is said he used the handshake induction so often that by the end of his career no one actually wanted to shake his hand. Many of his statements have been quoted as his definition of hypnosis—all are worth reading. What follows is just one of them:

'A state of special awareness characterized by a receptiveness to ideas.'

Milton Erickson. (16)

Two things are worth noting from this succinct definition. He emphasises that the mind becomes receptive to the ideas that The Hypnotist presents. This can be interpreted as a person becoming more suggestible or more open to the ideas being presented to them when they are hypnotised. The emphasis Erickson places on awareness, rather than on being unconscious, inattentive or unaware is also interesting. It is in line with his thinking that hypnosis allows you to deal with the bigger beast in all of us, the subconscious mind

or what he called the unconscious mind. When hypnotised, the unconscious mind seems to assume more responsibility or come to the fore. The unconscious regulates all of your bodily processes, stores and manages your memories, every learning from every experience as well as the mental patterns and templates that allow us to function. This part of the mind is intuitive. It can call up your potential and instantly change the way you think, feel and respond. By contrast the conscious mind is limited. It is logical and linear in its approach to problem solving. It is the here and now. The model is essentially a dissociative model, whereby the conscious mind is dissociated from the unconscious mind, preventing it from interfering with the functioning of the suggestions.

Erickson was not interested in talking to the conscious mind. Neither should you be when you are in the process of hypnotising. Aim to communicate directly with the automatic unconscious.

Finally, consider one of the most well known and quoted definitions from another innovator and perhaps the most influential hypnotist of all time, Dave Elman (1900—1967):

‘Hypnosis is a state of mind in which the critical faculty of the human is bypassed, and selective thinking established.’

Dave Elman. (17)

Elman refers to a state where ‘... the critical faculty of the human is bypassed...’

So what is the critical faculty? It does not seem to correlate to any physical part of the brain or neurological process. It is more conceptual—think of it as being a filter between the conscious mind and unconscious mind. The conscious mind can be thought of as having certain characteristics. It is rational, logical, it is limited and it is typified by inductive thinking—proceeding from certain facts to a logical conclusion. By unconscious mind I mean everything else other than the conscious critical mind—all of your memories, every learning, resource, pattern and template. It is creative, intuitive and seemingly unlimited.

The critical faculty can be thought of as your sense of judgement, or, perhaps more accurately, your sense of control. The bit of you that thinks it knows what reality is. It thinks it knows hot from cold. It thinks it knows that a mop is not the person you are in love with. It thinks it will hurt if you stick a needle through your arm. It thinks that you could lift your

feet up if you wished to. It believes you do know your own name.

Bypassing the critical faculty does not establish hypnosis, but it does represent, as Elman put it, the 'entering wedge.'

When the critical faculty is bypassed, your sense of judgement, inductive reasoning and logical faculties become suspended or inattentive. How inattentive and for how long they remain suspended is reliant on the attitude of the subject and the ability of The Hypnotist. When attitude and ability are both conducive to hypnosis, the unconscious mind of your subject becomes dominant, assumes responsibility and with further direction from The Hypnotist selective thinking can be established swiftly. According to Elman, selective thinking is that which you believe wholeheartedly.

By selective thinking I mean a style of thinking where inductive reasoning is suspended and the mind becomes locked around an idea. When this occurs The Hypnotist's suggestions will be listened to by the unconscious uncritically. They will be acted upon uncritically. Automatically.

That does not mean the unconscious cannot refuse to go in your direction; it can. It does not mean the critical faculty will continue to remain bypassed; it can pop back into play. However, as The Hypnotist, understand and be clear that to all intents and purposes hypnotising someone results in their unquestioning acceptance of the ideas, suggestions and directions delivered by The Hypnotist.

Theoretically the critical faculty can be bypassed in a variety of ways quite naturally without hypnosis. Experiencing confusion, shock, high emotion, information overload, being drunk or high on drugs, laughter, play and performance are all common instances where our sense of judgement and logic can be temporarily suspended. It is the rabbit-in-the-headlights moment. Whatever follows is generally driven by our unconscious, instinctive, automatic mind. The Hypnotist can create such moments artificially and utilise the result to establish selective thinking.

It is useful to note that in none of these definitions is there any mention of sleep or relaxation. That is because hypnosis is not sleep and does not require the subject to feel relaxed. What is emphasised is that when hypnotised the subject's attention narrows and becomes fixed around selected ideas or a single idea. They experience their behaviour as a happening rather than as an act of doing. Wider environmental stimuli are often ignored.

Recently a fresh model of hypnosis and the mind entranced has emerged from the Human

Givens pioneers Jo Griffin and Ivan Tyrrell. (18)

The model they have presented is not a multiple-minds model and does not require a state. It simply talks of triggering the ‘orientation response’ which fires up the ‘reality generator’ to which The Hypnotist provides content in the form of suggestions. What they refer to as the reality generator can be thought of as the dreaming brain, the mechanics of which are responsible for our dreams and active during the REM (Rapid Eye Movement) phase of sleep. One of the functions of dreaming is to discharge unresolved emotional arousal. In other words it allows us to complete emotional ruminations of the day through the metaphoric imagery and connections of our dream. Its other key function is to update our instinctive templates or behavioural and emotional responses. In other words the REM phase is when we lay down learnings. Whenever we act without conscious effort we are reliant on pattern matching, going back to an earlier learned response or behaviour that was set in REM. So when we act instinctively we are, in effect, acting on a post-hypnotic suggestion. In the same way when a hypnotised subject acts on a post-hypnotic suggestion given by the hypnotist they will do so with the same effectiveness, immediacy and instinct they do other unconscious behaviours.

So when we put someone into hypnosis we are simply activating the same processes that the brain activates during dream sleep, including the reality generator—this is what makes it so effective. Although the Human Givens definition avoids conceptual issues with dissociation and state, it is, in reality, a special process model of hypnosis. The detection of this special process could be used to distinguish between unhypnotised and hypnotised modes of operation. However, like the state-based view, evidence for such a special process is lacking.

For your purposes, as The Hypnotist, it is useful to keep these definitions in mind. Revisit them in the light of your experience. Read the work of those who coined them, taking into account that their ideas are a product of their time.

As interesting and desirable as it may be to understand exactly how hypnosis works, as a practitioner there is no need to let the lack of consensus amongst theorists stop you from getting started.

What remains important is the subjective experience of the subject. They alone can qualify if the effects of the suggestions were accompanied by the classic suggestion effect mentioned previously. This characteristic sense of involuntariness is common to many schools of thought regarding hypnosis.

The mind exists as a model. Hypnosis exists as a phenomenon. We must use a conceptual

model to describe how it works. Hypnosis may not have the reality of a house brick but that is of no consequence to you. The fact is you can hold any of the major views about hypnosis and still be a good hypnotist.

Over the years I have considered many definitions. Currently I define hypnosis as follows:

Hypnosis is a social construct that causes the cognitive processes of automatic imagination. Hypnotic responses are defined by their subjective sensation of automaticity or involuntariness because they lack the knowledge or feeling of intention.

For practical purposes as The Hypnotist, think of it this way:

The Hypnotist presents suggestions to the subject and the subject experiences the effect of those suggestions with a sense of involuntariness.

So understand, your job is to turn ‘a doing’ into ‘a happening.’ As The Hypnotist you are presenting ideas and giving suggestions. You are doing this to your subject’s automatic unconscious mind and it is receptive to the ideas you are presenting. Believe, want and expect that it will interpret them and act on them with a genuine unconscious response.

What is Pain?

Obviously we all know what pain is. However if we are to help alleviate it, it is useful if we can first define or describe pain. This allows us to examine what makes it up and shape our intervention accordingly.

Pain is a subjective experience, that normally feels unpleasant, like something in the body has been, or is being damaged or destroyed; that feels like a threat to or interference with one's ongoing functionality and health; and that is associated with negative emotions, such as fear, anxiety, anger or depression.

This definition does not require you or anyone else to objectively demonstrate the pain. It does not require that an association be made between the unpleasant sensation and tissue damage. Also it does not eliminate the possibility that the pain is because of tissue damage.

The Arrow Technique places importance on the subjective 'felt experience' of the person in pain.

This definition acknowledges that pain contains several elements

1. Bodily sensation with qualities like those experienced during or after tissue-damaging stimulation.
2. An experienced threat or interference with functionality associated with this sensation.
3. An emotional feeling of unpleasantness and/or other negative emotions.

In other words, pain has a sensory and an emotional component. These two components are often intertwined. (26)

One of the keys to relieving pain is to disentangle these two components, the sensory / physical from the emotional / mental. Without the emotional element, what is left does not hurt as much or at all.

Acute Pain

Acute pain is pain that is of recent origin. It can last anything from seconds to months. The new pain from a recent injury is acute. The immediate pain encouraging you to move when you stub your toe or burn yourself is acute. It is a type of pain that typically lasts less than 3 to 6 months, or pain that is directly related to soft tissue damage such as a sprained ankle or a cut finger. Acute pain has immediate value. Acute pain keeps us from harm, it prevents us from destroying ourselves. It ensures we tend to, correct and take care of ourselves when healing from an injury. It stops us from overdoing it. With the appropriate care and treatment acute pain should subside as healing takes place. When pain lasts beyond that time that might be considered reasonable for a complete recovery, it is considered chronic.

Chronic Pain

Chronic pain can last for weeks, months, even years. Generally, it's diagnosed after three to six months of pain. This type of pain can continue even after the injury has healed or the illness that caused it has gone away. In some cases, the pain comes and goes. In other cases it is persistent and unrelenting. It is a pain signal that continues past the point of us needing to be alerted to something, such as danger, to tend to a wound, or move away from something. It is pain that has not responded to appropriate medical care. Its intensity and the disruption it causes can be overwhelming.

Types of chronic pain include: recurrent headaches; continuous facial and jaw pain; persistent neck and shoulder pain; low back pain; arm, wrist and hand pain associated with carpal tunnel syndrome; the leg pain of sciatica, pain linked to osteoarthritis and rheumatoid arthritis, the continual, intense burning pain and hypersensitivity of the skin, muscle and nerves sensitivity associated with some nerve-damage conditions; the multitude of symptoms associated with fibromyalgia, the ringing of tinnitus, the cramping of IBS and the soreness of unexplained pelvic pain.

Suggestion and pain control

Hypnosis is a tool that offers considerable leverage in changing behaviours and experiences related to pain. Tales of quite miraculous relief from pain have been associated with hypnosis from antiquity to the present time. The good news is that peer-reviewed scientific research backs up some of these claims. (27)

A meta-analysis of controlled trials of hypnotic analgesia indicates that hypnosis can provide significant relief for 75% of the population. (28)

In Drs. Patterson and Jensen's review of the use of hypnosis for control of pain, they concluded that hypnotic techniques for the relief of acute pain (an outcome of tissue damage) are superior to standard care, and often better than other recognized treatments for pain. In the case of chronic pain, Patterson and Jensen's review found hypnosis to be consistently better than receiving no treatment, and equivalent to the other techniques that also use suggestion for competing sensations, such as relaxation and autogenic training. The effect is largest for those who are highly suggestible, but is also relatively large for moderately suggestible people. Because hypnotic pain control includes a placebo element even low suggestible people can experience a reduction in pain through suggestive techniques. (29)

In addition to the biological element, the experience of pain includes subjective and cognitive components that lend themselves to hypnotic modification. Pain has a sensory component and an affective component. The sensory component pertains to the intensity of the pain experience. The affective, concerns the unpleasantness of the pain. That is the individual's subjective level of distress, which may be driven by conditions that fluctuate over time. The Arrow Technique begins and ends with an enquiry into these components. Hypnotic suggestions can affect one or both components. Without pain it is difficult to maintain unpleasantness. With no unpleasantness it is difficult to register sensations as painful.

The placebo effect can be thought of as an indirect suggestion for an effect. However it is normally deceptive, in that the person taking the placebo, is normally not made aware it is a placebo. This mostly limits use of placebo to research. The effects of hypnosis can be thought of as a non-deceptive mega-placebo. We are going to tap into a natural ability we have; to ignore and let go of unnecessary pain. We will use a variety of strategies to achieve this.

Maladaptive thinking is the central mechanism in maintaining chronic pain and therefore its management. The main kinds of maladaptive thinking are catastrophizing, meaning-making and self-suggestion. These all have the potential to foster hopelessness and to intensify the experience of pain. These thinking styles can be changed or decreased by alerting the client to their nature and impact and prescribing new strategies. Or they can be dealt with tacitly via the intervention itself.

In this sense suggestion and The Arrow Technique supplies a kind of multi-modal cognitive strategy for engaging in effective ways of experiencing, using thoughts and images consistent with the therapeutic goal of being free from unnecessary pain and discomfort.

Being The Hypnotist

To be a great hypnotist it is of key importance that you become The Hypnotist. Not a hypnotist. Not someone who knows a bit about hypnosis, but The Hypnotist. You must express absolute confidence, congruence and expertise in your skills, knowledge and abilities. In the beginning this takes a certain amount of front. Pretend and master it. Believe you are one of the best, believe you are a natural and behave like you are.

Believe your subject is a wonderful hypnotic subject and that their imagination is good enough. Want and expect them to go into hypnosis and do what you tell them.

People who are in pain, do not tend to ask too many questions about your qualification to assist them. They just want to know if you can help them. It is not that they have to believe in you having special powers. They just have to take you seriously enough to actually engage with your instructions. So adopt the attitude that you can help them. Mean what you say and intend to make a difference. This is important. You need to be able to look people who are suffering in the eye and say, 'I can help you.'

Suggesting that you can sometimes help people be free of pain, to people who are in chronic pain of course requires some sensitivity. It also requires you to speak with conviction, that the result is possible. Have good intentions and cultivate the attitude that this change can happen. The truth is they have the resources they need and that you can facilitate this.

The Set Piece

What follows are two simple exercises either of which, The Hypnotist can ask the subject to participate in before using The Arrow Technique. If the plan is to use The Arrow Technique in a hypnotic context with an induction, then this exercise would typically precede The Jacquin Powerlift induction or act as the induction itself.

In normal practice we use only one of the examples given here, that is The Jacquin Fingerlock. Magnetic Fingers makes up the first part of that technique, so is included here for completeness.

The Jacquin Fingerlock gives the subject a positive experience and elicits a strong emotional response.

It is possible to go straight into The Arrow Technique from this point, without using an additional induction.

There are just so many good reasons why The Hypnotist should be able to confidently do a Set Piece. So learn them well. Understand the principles and applications. Use them.

- * The Set Piece is often used to test how responsive to suggestion the subject is.
- * The Hypnotist can observe the subject taking instructions, get feedback and assess their suitability as a subject during The Set Piece.
- * The subject experiences The Hypnotist's power of suggestion and influence. If the subject experiences something they consider out of the ordinary they gain confidence in both the skill of the hypnotist and their own ability to be hypnotised—they let go and become more fascinated with the process.
- * Likewise The Hypnotist gains confidence in the subject.

* The Set Piece fires up the imagination, focusses attention and builds expectation in the subject that they are about to be hypnotised or are able to change.

* Finally The Set Piece can be used as the induction into hypnosis. This is perhaps the most powerful and overlooked application.

To begin it is fine to simply say:

‘Let’s try something’, or ‘Let me show you something interesting’ or call them an **‘Exercise in concentration’** and proceed.

Deliver The Set Piece with a tone that is bright and upbeat, a manner that is confident and commanding and keep things moving along at a fairly rapid pace.

Find your own way and ensure that your attitude communicates that you are confident, knowledgeable and expecting to hypnotise and to help.

It is important that your subject does not see The Set Piece as something they should be trying to battle or resist. Equally you do not want them to pretend. Make sure they understand this. Clarify what their role is. For example if you were going to ask them to imagine their fingers were like magnets so that they come together and touch automatically, you can say this:

‘I don’t want you to push them together or try to keep them apart; I just want you to concentrate, imagine your fingers are magnets and your body will respond.’

If they follow your instructions, concentrate on the ideas you are presenting and genuinely use their imagination, then they are very likely to do well in The Set Piece and be set up perfectly for the hypnotic induction if you choose to use one, or The Arrow Technique itself.

Magnetic Fingers

In this exercise you are going to make the index fingers of your subject move together like magnets. You will find nearly everyone achieves the objective result. Your job is to pay attention to the differences in the subjective response, keep an eye out for signs that the subject is feeling the involuntariness.

‘OK, let’s try something. A simple exercise to fire up your power of concentration. I’d like you to place your hands out in front of you like this.’

‘Now can you clasp your hands together; palms together and thumbs crossed; nice and tight.’

‘Now bend your elbows like you are making a desperate prayer. You can make one while you are there if you like.’

‘Now put your first fingers, your index fingers, straight up, about an inch apart. Look at the gap between them, because in a moment they will come together and touch; just like they are magnets; that’s it, they are starting to twitch; closer and closer and as soon as those fingers touch; you can allow your eyes to close and relax. Now take a nice deep breath in and as you breathe out let your hands drift down and relax.’

‘Open your eyes. Brilliant; that shows me you can concentrate.’

Ninety-nine per cent of people will do this successfully. You should aim to get their fingers together swiftly, as quickly as two seconds and no more than twenty seconds. If they cannot do it within this time, do something else. The reason this exercise is so easy to succeed in is because the effect of the fingers moving without conscious effort is heavily reliant on the physiology of the hands. Try it out and this time squeeze your hands together as you watch your fingers. As the tendons in the other fingers tighten it causes the tendons in the index fingers to tighten and they come together quite automatically.

The most common presentation of this exercise involves a demonstration and explanation of what is going to happen and a request to ‘Squeeze the hands and all of the fingers together except the index fingers.’

In most cases you will get away with this approach. However it does make it kind of obvious that some physiological principle is at play and takes the fun and surprise out of it. So to avoid making it so obvious do not mention squeezing the hands as you give the suggestions about the fingers. Just set things up properly and use suggestion.

Do what you are purporting to do and make it happen with your words and influence to give them a real sense of involuntariness. Be The Hypnotist. This is often your first real chance to do something with your subject; make it count.

You will see from the scripting above that the tension is placed into the hands in the set-up. This is important because if the subject just has the fingers loosely interlocked it is much less likely they will succeed in experiencing their fingers moving, so ensure that they follow your instructions. If they have not, then say it again making it absolutely clear what you want them to do. Demonstrate with your own hands as you go through the instructions. Then as soon as the hands are set up there should be a change of pace and a direct suggestion from The Hypnotist about what is going to happen. State the outcome that they will touch first, rather than beginning by describing the magnets, as otherwise they might touch before you suggest that they will do so:

‘... Because in a moment your fingers will come together and touch, just like they are powerful magnets.’

You should increase the tempo and forcefulness of your delivery at this point. When using this technique and any similar exercise, even though you know this will work, your mindset as The Hypnotist should be that you are making this happen with your suggestions. Then you will be congruent and convincing in your approach. Your words and actions will create the effect.

Although simple and easy to dismiss as a trick, you will be surprised at just how much some subjects react to this test. Wide-eyed expressions and exclamations of surprise that what you are saying will happen is happening are common.

The Jacquín Fingerlock

In this brilliant extension of magnetic fingers, we add suggestions to elicit an emotional response from the subject. This experience is then linked to the fingers sticking together. This can be pushed further as a ‘challenge suggestion’, where the hypnotist encourages the subject to fight against the suggestion, essentially bypasses the need for any further induction. This is our preferred technique prior to delivering The Arrow. It allows the client to have a positive and novel experience, as well as to engage emotionally. This emotional engagement is very welcome when we wish to give suggestions the best chance of being engaged with. It ends with a direct suggestion regarding the outcome of the session.

The extension of the Magnetic Fingers technique outlined here is something we frequently use in group sessions of hypnosis, sometimes with hundreds of people at once, often, early on in his presentation as a simple demonstration of real mind power. There is no need to mention hypnosis. Presented in the way it is outlined here it is something of a ‘feel good’ exercise that encourages the client to engage emotionally while having a novel experience. This can be quite a profound experience for some people and leads into a moment where the assertion that they may be able to be free of pain becomes a more plausible possibility.

By now you understand why Magnetic Fingers works from a physical perspective. The fact is there is a physical reason why our index fingers come together when we do Magnetic Fingers. However there is no physical reason why they should stick together – by that I mean become difficult or impossible to separate. This extension of the technique achieves that. It turns Magnetic Fingers into a Finger Lock and if you wish into a Hand Lock. If you have achieved such a lock then you have achieved hypnosis. The mind has locked around at least one idea, the idea that the fingers or hands are stuck. The subject experiences the classic suggestion effect - a sense that things are happening non-volitionally.

To present it you first of all proceed in the same way you do with Magnetic Fingers, to the point where the subject closes their eyes. You then change tack by asking them to imagine something, something that engages them emotionally. In so doing you dissociate them from their body, while providing more and more direct suggestions that their fingers are sticking and eventually that they are stuck. Only then do you begin to challenge them a little and test your work.

‘Can you put your hands out in front of you? Can you clasp them together, nice and tight? Bend your elbows like you are making a desperate prayer. Point your first fingers up, an inch apart.

Look at the gap between them, because in a moment they will touch.

Just like they are magnets. They are already moving. Imagine they are magnetized and they are going to touch, and when they touch, close your eyes.

Now squeeze tight.

Go out into a fantasy and think of the person who is dearest to you on this planet. If there is more than one, see more than one. See their eyes, see them smiling. Hear their voice, hear them laughing. Feel what you feel for them and notice where those feelings are.

Now as you feel that, and as you listen to me, your fingers are becoming stuck and bolted together, they are super-glued together they are stuck.

At this point you could simply instruct them to hold on to those good feelings and open their eyes when they are ready to let go of their pain. Then proceed with an induction or The Arrow Technique. At this point you have not given a challenge suggestion. If you wish to use the challenge phase then continue as follows:

(TEST 1)

When you realise they are stuck raise your hands a little higher, so that I understand.

(TEST 2)

Locking, sticking tighter and tighter; to the point you can try and pull them apart and find they stick even more.

(TEST 3)

That's right. You can try and pull them apart and find they remain stuck fast.

(TEST 4)

You may even find they start to shake or vibrate, it is nothing to be concerned about.

(TEST 5)

When I clap you can open your eyes and look at your hands stuck there, as if they are someone else's hands.

Now you can hold on to those good feelings, I am going to touch you fingers and when I do your hands will unstick and you can relax completely.'

If pain control is our aim then we could link the undoing of the hands to the client being primed to let go of any unnecessary pain.

Now you can hold onto those good feelings, as you mind prepares itself to let go of any unnecessary pain or discomfort. In a moment I am going to touch your fingers and when I do your hands will unstick and you can relax completely, ready to be hypnotised and make those changes that will give you complete freedom from any unnecessary pain'

Induction

Induction is the generic term used to describe any procedure used to hypnotise. In essence it is a suggestion to become hypnotised. Almost anything you do or say can be used as an induction. A diverse range of procedures have been used throughout the history of hypnosis, from bizarre esoteric practices such as clashing gongs and flashing lights, to simple repetitive suggestions for relaxation. Whether it is commanding someone to sleep, swinging a watch or waving a cucumber, the common ingredient is the association with the word hypnosis. The procedure, per se, is not that important, but whether or not the subject perceives it as part of an appropriate cue for displaying hypnotic behaviour is. For this reason the number of possible inductions is limitless and that is why they continue to proliferate. There are thousands of them to choose from. Some inductions are direct; some are indirect. An induction can be rapid, instant, or slow and progressive. It can be overt or covert, verbal or nonverbal.

The induction outlined here is a rapid, overt and verbal induction. It has a physical element to it too.

Although being flexible in your approach is desirable, you will be more effective by becoming extremely accomplished at just a few inductions, rather than simply knowing lots of them. It can be fun to learn more, of course. However, continuing to seek out new inductions in the hope that finding the ultimate induction will make you a better hypnotist is normally a fruitless effort. Ultimately it is The Hypnotist that does the hypnotising not the procedure itself.

The inductions you decide to master and use should be suited to the setting and circumstances you plan to use them in.

Although no particular induction is better than another, in terms of how the subject would fare on a subsequent test of suggestibility, it is worth considering your subject's unique attitudes, preferences, abilities and expectations as well as their condition. Some of these variables might have an impact on how well they respond to the task you are presenting them with. Often the pre-hypnosis discussion will yield useful information that will help The Hypnotist in refining their induction for best effect without necessarily changing it altogether. Be sensitive to the following variables and flex your approach accordingly:

- * Attitude. What is the subject's attitude? Do they think their role is to challenge or is it entirely passive?
- * Preferences. Have they expressed a preference for relaxation? Do they fear giving up control?
- * Abilities. Do they have the ability to think, visualise, fantasise and absorb themselves?
- * Expectations. What do they expect? Have they experienced hypnosis before?
- * Condition. Does their condition prime them for or prevent them from doing certain things?

We prefer inductions that contain a mixture of instructions and suggestions. Everyone can follow instructions. They are a request to do something. Suggestions may sound similar to instructions but they contain a cue, suggesting that the response that follows will happen automatically; experienced with a sense of involuntariness to some degree. Suggestions provide The Hypnotist with feedback during the induction itself, showing early on whether the subject is responding by experiencing the classic suggestion effect.

We prefer inductions that provide a novel situation for the subject. It makes it slightly ambiguous how they should proceed and what is being asked of them. Because of this ambiguity in some aspect of their role, they wait for further instructions and suggestions. This situation provides The Hypnotist with options about how best to proceed. We refer to this situation as having leverage. For example, if during the induction The Hypnotist was to lift the subject's arm up by the wrist, this provides options about what might happen next. It could be that the arm feels heavy or relaxed and would drop down if The Hypnotist let go. It could move automatically. It could lock into place. Sometimes the arm will feel naturally more inclined to one of these outcomes. The Hypnotist can then tweak the induction and make suggestions that will exacerbate that condition, such as if it feels heavy, make it heavier. Alternatively The Hypnotist can choose to make suggestions that the arm will adopt some other condition, perhaps the opposite. For example, if it is light, make it heavy. If it is stiff, make it floppy and relaxed. If it is still, make it move.

Keep in mind that if your subject has responded well to The Set Piece then they may

already be hypnotised. The bottom line is, they are already responding to your suggestions automatically. If you have suggested the subject's hands are stuck, despite your instruction that they try and pull them apart, then they appear to have already established selective thinking around at least one idea—they cannot pull their hands apart. Their unconscious is seemingly receptive to the idea you are presenting to it. The effect The Hypnotist's suggestions have created, feels out of the voluntary control of the subject.

It may not have occurred to the subject that they are hypnotised or even that the effort to hypnotise has begun. In this sense the induction that follows The Set Piece is a way of intensifying the subject's ongoing hypnotic behaviour rather than creating it.

Confidence, certainty and a matter-of-fact attitude that the person will get hypnotised go a long way to using these techniques successfully. Some hypnotists get this right from the start; others have to do their best to develop this character. It is of course helpful if there is a sense of trust between The Hypnotist and the subject. The discussion should have taken care of this. The Set Piece should have ratified that.

Jacquin Power Lift

This induction combines confusion, rehearsal and a pattern interrupt. It results in catalepsy. It is rapid and direct, and reliable and flexible. It can be used one-to-one or with small groups in any setting. These rapid inductions success is reliant on your intent and expectation, not the technique itself. So think about how you are communicating that. This is our preferred induction prior to using The Arrow Technique, especially if the subject is seated or standing somewhere where their elbow can be supported. That said, it can also be done without elbow support. Obviously be mindful of the subjects condition, just in case they have a condition that the physical demands of this induction could aggravate. It is a perfectly lead into The Arrow Technique and also allows us to link the lowering of the arm to the ratification that the work has been done at the end of the technique.

‘OK, I’m going to take you into hypnosis. I am going to show you what I am going to do then I am going to do it.’

Point at their hand and say the following:

‘In a moment I am going to borrow your arm. Is that OK with you? I will reach over and pick up that hand, and as I lift it up like this...’

Demonstrate by reaching over with the thumb and first two fingers of one hand, taking hold of your own wrist and lift it up until your palm is facing forward. Close your eyes as you do this to make clear what you want the subject to do:

‘... As it lifts to about here, I want you to allow your eyes to close. As I push it down (demonstrate) your eyes will open and you can just relax.’

Demonstrate again by opening your eyes as the hand moves down. Then give a second demonstration:

‘As I pull your arm up, your eyes will close, as I push it down, your eyes will open and you will relax even more. Is that OK with you? So can I borrow your arm?’

Reach over and pick the arm up by the wrist bending the elbow. There is no need to pull it up immediately; perhaps pause a moment or two, to look for any of the signs of hypnosis as they begin to focus on that idea.

‘As I lift it up, you can allow your eyes to close.’

Note the permissive language ‘... you can allow your eyes to close’, It is an instruction. They will close their eyes.

Hold their wrist in a way that makes them wonder whether you are holding it or not and whether you are lifting it or not. A light touch, sometimes lifting one of the fingers that is holding their wrist, creates a kind of ambiguous touch that creates some confusion.

Attempt to get to the point where their hand is ‘held’ just by your having one finger remaining in contact with the back of their wrist.

Then re-grip and push their arm down slowly almost as if you were meeting some resistance and it was slightly difficult to push the arm down as you say this:

‘As the arm goes down you can allow your eyes to open and relax even more.’

This added bit of tension is easily achieved by tensing the muscles in your own arm. It gives the impression to the subject that their arm is feeling slightly strange.

Repeat, adding some pacing comments and shifting from permissive language to a more direct tone and commands. This time the instruction is replaced with a suggestion that the ‘eyes will open/close’:

‘That’s right; the arm lifts and your eyes will close. I push it down and your eyes will open as you continue to relax.’

Go to lift the arm up for a third time and use the following shift in your language to suggest

that the arm is lifting and the eyes are going to close automatically. Due to the built-in rehearsal it probably will lift automatically or with very little assistance.

‘This time as the arm lifts and your eyes close; feel that wrist getting stiffer.’

As you mention their wrist getting stiffer, squeeze it a little to encourage tension.

With your free hand, poke them in the forearm muscle with the tip of your finger and say the following:

‘Feel the forearm muscle tightening.’

Poke the elbow joint:

‘Feel your elbow stiffening.’

Poke them in the biceps and shoulder as you say:

‘Feel the biceps stiffening... into the shoulder. Every muscle and fibre...stiffer and stiffer as it hangs on that wire...’

As you mention the wire, tap your index finger on the crease of their wrist:

‘Each word that I say and every breath that you take will take you deeper and deeper into hypnosis.’

Repeat the idea that the arm is getting stiffer and intensify the situation. You have created a situation of leverage. The subject is now in hypnosis and in an unusual situation. Their eyes are shut and they have their arm stuck in the air. You can simply leave their arm there and then when you have completed The Arrow Technique, link the arm coming down to the completion of the work.

Or you may wish to link the arm lifting or falling or moving toward their face to whatever you wish. For example, the strongest next step might be to say this:

‘Feel that arm getting lighter and beginning to lift up. That arm is lifting off the chair now, getting higher and higher. Just like your head is a magnet and your hand is a powerful magnet. When it touches your face you will go into a profound state of readiness to make these changes.’

If you do not think you can encourage their hand to their face then you could simply link their hand falling back to the chair to their going into a deeper state.

‘In a moment I will count to three and on “3” that arm will slowly drift down, all the way down on that imaginary wire. Your eyes will remain closed this time as you continue to relax and drift deeper into hypnosis. As the muscles relax you will relax; as that arm drifts down you can drift down. 1, 2, that’s it all the way down now, 3.

That’s right; drifting down to a comfortable resting position as you drift deep inside, deeper and deeper.’

Getting Deep Now—Intensification

Once you have suggested hypnosis you can intensify the situation the subject is in and make their new reality more vibrant. This process is commonly referred to as deepening. It involves The Hypnotist suggesting to the subject that they can go into hypnosis even more. More deeply. More profoundly. More intensely. Effortlessly. Automatically. Comfortably.

For some time I pondered whether you really have to give the subject time to get comfortable with it all—surely comfort is a subjective enough opinion. If they are ‘in’ surely they are ‘in’. However, in practice it seems to help things along if, after you have suggested hypnosis, you spend a little time ensuring the subject knows they are doing OK, that they are safe and well and that you are happy with them. This gives the subject a chance to get a sense of what this new hypnotic world is like. The easiest way to do this is to tell the subject to ‘Go deeper’.

A common view amongst many hypnotists and the public is that there are many ‘levels’ of hypnosis. It follows that the metaphorical associations of ‘going deeper and deeper’ when coupled with ambiguous ideas about ‘going into’ hypnosis result in much theorising about how deep someone is. Strangely, given all the focus on depth, many hypnotherapists continue in their work without knowing if their subject is hypnotised—the reason being that they do not test for hypnosis, light or deep, instead assuming that because their subject is looking relaxed and has their eyes closed that they are hypnotised.

They might be, but they might not. Rather than risk exposing this potential illusion they do not actually ask for a direct sign that hypnosis is established.

They do not do anything that would evoke the classic suggestion effect. There is no excuse for this—fear is not an excuse a hypnotist should have. Test your work. This is easy to do. Give the subject a suggestion and see if they respond to it—unconsciously. If the only suggestions you give are to ‘Relax and go deeper’, at least look for signs of that happening and ways you might test that. The Jacquin Fingerlock and The Jacquin Power Lift Induction both have suggestions built-in, that test if the subject is responding by providing feedback. So be alert and sensitive to that feedback.

It can be argued that all this discussion of levels and depth just causes confusion. There is just one level: hypnosis. This is when the subject is responding to a suggestion with a sense

of their response being automatic or involuntary. It will make you more effective as The Hypnotist if you think of it this way. The subject is either hypnotised or they are not. You are either having the response to your suggestions driven automatically by the unconscious mind, without critical inductive reasoning, or you are not. Personally I do not think the idea of levels of hypnosis or depth are valid or necessary concepts to consider when trying to understand how hypnosis works. I simply think of suggestions for deepening like any other suggestion, no different to ‘magnets’, ‘lighter’, ‘stuck’ or ‘sleep’. I do accept that they are very useful suggestions though. They help shape the subjective experience of the subject and this can in turn help improve their responses.

Immediately after the induction, begin suggesting deepening. Look and feel for feedback. By all means suggest and send your subjects ‘Deeper and deeper and deeper’. Send them ‘Deep asleep’, ‘Deep inside’ and ‘Ten times deeper’. When you see signs that this is happening, give suggestions that intensify those responses. If you see the subject’s head dropping forward as their neck relaxes, say, ‘Neck relaxing, shoulders relaxing.’

Regarding hypnosis, the phrase ‘Deeper and deeper’ is embedded in the psyche of most people, just like the word ‘Sleep’ is. Just because you know hypnosis is not sleep and that they may not be going down through levels, that is no reason not to use these words. Imagine you have a clear line of communication with their unconscious, however you perceive that. There is no need to fuss over what the subject is thinking—they have an onboard model of hypnosis and whether they realise it or not they know what to do! Their unconscious, automatic, pattern-driven abilities are becoming easier to talk to. You are no longer talking to their conscious mind. Ignore it like it does not exist. Your goal is to deliver a suggestion to intensify the condition they believe themselves to be in. There is no need to spend a long time doing this. On stage and in the therapy room I might spend a couple of minutes doing it. In an impromptu or therapeutic situation it might be just a few seconds to a minute. In deepening you begin to direct your subject deeper into the state they are now in and do this in a pleasant way. There really is nothing to fight. The suggestions you are giving them are easy to follow and not objectionable in any way.

You can continue to deepen each time you re-awaken and re-hypnotise the subject. Every now and again just give the subject encouragement to go deeper. If you pull this off properly, then you are ready to move on from intensifying things to directing their unconscious further and shaping their reality however you wish.

There are four main ways of deepening: links, loops, chains and bombs.

Links

A link connects one thing to another. Use suggestion to connect things with the idea of deepening.

‘In a moment I will ask you to open your eyes and then close your eyes.

When you close your eyes that is your signal to go twice as deep into hypnosis. OK. Open your eyes. Now close your eyes and double that feeling; go twice as deep.’

This approach is known as ‘fractionation’. You first do an induction, then reorientate the subject back to the room, then send them back into hypnosis. You go over this a few times, each time adding more suggestions. It works wonderfully. Do this and manage their gaze with yours, or by holding up something for them to look at.

Keep it short and peppered with suggestions that will turn things up a level. Another simple link is to associate going back into hypnosis with going twice as deep as before.

‘Each time I say the word “Sleep” you will return into this state of hypnosis, going twice as deep as before.’

Here is another example:

‘In a moment I am going to touch the back of your hand and it will begin to lift up toward your face. As it (touch hand) begins to feel lighter and lighter and lift up all by itself, you can enjoy a wonderful sense of well-being as you continue to drift deeper into hypnosis. The closer it gets to your face the more relaxed you become.’

This is a little bolder, because you are about to test your work by giving them a suggestion that you are expecting them to respond to automatically. When their hand lifts, associate

it with going deeper. Give it a physical target or point-in-space to aim at, like their head or face; this builds some expectation that when it gets there they will go even deeper still. Indeed, when using any technique with some movement or repetitive behaviour, associate the process and completion of the process to something you want the subject to do or feel. In this case that would be depth.

Loops

These statements create a feedback loop that will intensify hypnosis. They create associations that feed off of each other. Some are short-lived loops and some are continuous.

‘Go deeper as you notice your eyes flickering; as you drift deeper and deeper they will flicker even more.’

‘The deeper you go the better you feel and the better you feel the deeper you go.’

‘Even as you wonder how deeply you have drifted, you can continue to drift down deeper relaxed.’

‘As I rock your shoulder (with one hand on the subject’s shoulder rock their body gently back and forward) go deeper and deeper. That’s right, your legs will support you even as you continue relaxing.’

Chains

‘Every breath you take will send you deeper and deeper.’

‘Every word I say will send you deeper and deeper.’

‘Every beat of your heart will take you deeper and deeper to sleep.’

‘Every number I go past, every breath you take and every beat of your heart is doubling the relaxation.’

I am sure you get the idea. If you can associate something in ongoing experience that is not going to stop, then you have a chain—a strong loop. This is also a good time to begin giving some procedural instructions that makes clear what you expect of them.

Bombs

Sometimes immediately after suggesting sleep, a single word or phrase is enough to send the subject flat out into what appears to be a significantly deeper and more profound experience.

‘Sink, drift, melt, bliss; go there now.’

These are typical commands I will give at this point. Often they are all I need. The effect is instant and complete. Combined with some physical encouragement such as a finger-tap on the head, or a gentle rock of a shoulder, or the body or one hand gently applying some pressure to a shoulder, many subjects just get the idea and dive into whatever those words mean to them. It is beautiful when this happens. These short phrases have a similar effect:

‘Drift inside, just like other times and places you have let go completely.’

‘Let go, now.’

There is nobody wanting anything and nobody is expecting anything.

Double that feeling now.

Drift into your own private place of hypnosis, a blissful space, nothing to do for a time.'

The Arrow Technique

The Arrow Technique originally started out as Freddy Jacquín's impromptu chronic pain control technique. Time after time it gave miraculous results. Relief was provided within minutes. Often the relief is permanent. Even if the pain relief was temporary, it still provided a glimpse to the person in pain, that a portion of that pain was about their perspective, their relationship to it and this helped give them a sense of control.

Whenever you are going to attempt to help someone be free of pain, it is important to establish if the pain is of any use to them. Pain after all is initially a signal that something is not right. A rotten tooth still needs to be looked at, a burn still needs medical treatment. Many pains however, especially long-term chronic pains serve no useful purpose. So ask your subject if it is of any use to them; have they seen a Doctor about it, does it keep them safe in some way; perhaps preventing them from overdoing it?

If it is of no use to them, then make it your intention to give them total freedom from the pain.

If it is of some use, perhaps it stops them from lifting too much, then leave them with enough of the pain to act as a reminder, and keep them safe. Make it your intention to get rid of any unnecessary pain and discomfort.

The technique is easily adapted to use with what could be described as emotional pain and other things people wish to stop feeling. Use The Arrow technique to give relief from these long-term chronic feelings and thought patterns.

The Arrow can be used to get into resourceful states.

Once you have learnt the technique, you will see it is easily self-applied, helping you change the way you think, feel and respond in life.

Core Principles

The Arrow Technique combines a variety of principles and strategies. They add up to something that can provide quite dramatic results.

- * We let those we work with, gauge and communicate their own progress, using what is called a symptom scale. They are asked how severe their current experience of pain is. It is all we really have if we want to know how our subject is feeling. It's inherent lack of precision can be used advantageously at the beginning and end of The Arrow Technique.
- * In summing up his hypnotic approach, Freddy will often say 'Elicit an emotion and give a suggestion.' We suggest you invoke emotion by asking about the person who is dearest to the client if appropriate. This can be achieved by simply asking the question or by using The Jacquin Fingerlock, which requires them to do this. Our experience is that heightening emotion in this way prior to delivering The Arrow Technique helps the subject by focusing their attention and encouraging them to engage with the suggestions being presented.
- * Recognise that everything you say is a suggestion, and that any emotional spike is a good time to give a direct suggestion. So find the courage to make bold statements related to the intervention at such moments; statements that if true, would give considerable cause for hope.
- * The technique encourages relaxation. This is itself can provide relief.
- * The technique fosters a strong therapeutic alliance, one major element of which is 'hope of a recovery.' A strong therapeutic alliance refers to that felt bond between therapist and subject – it is the most powerful factor in the process of emotional and psychological healing. There are hundreds of studies that show that a purposeful, collaborative relationship between a therapist and the patient correlates with positive therapeutic progress. The most important aspect of effective therapy requires the patient and the therapist work collaboratively.
- * The technique involves imagined dissociation. This will help reduce discomfort. Some clients worry that they are not seeing things vividly enough. Yet research shows that no special imagination is required. So let them know that, however they imagine things is fine and encourage dissociation from their body, from the problem and from any conscious effort to change things.

* The technique encourages distortion and transformation of the client's representation of their pain. The use of symbolism is a common strategy people dealing with pain, one they often spontaneously engage in.

* Finally the technique uses direct suggestion especially at the emotional peaks.

Transcript - The Arrow Technique for Pain Relief

‘As you sit comfortably, I want you for a moment to focus on your breathing. I want you to pretend and imagine that you are breathing in calmness and breathing out tension. And as you continue to breathe like this, just focus on your body and if you notice any tension anywhere just let it go as you exhale. In your mind softly and slowly repeat the word “Relax” four times. As you repeat that word you can begin to relax. Let every muscle relax. As you do that, you may become more aware of the ideas and images, that drift into the mind automatically. And allow yourself to drift down deeper and deeper into that feeling. Allow yourself to drift deeper and deeper, and imagine that is happening all by itself, as you listen to the sound of my voice.

And now I would like you to allow yourself to have the experience of drifting out of your body, up out of your body; and drift away from your body and drift way, way up above your body, hundreds, thousands of feet above your body, leaving all physical ties behind, drifting way way up. Notice how it feels to be up there and as you look around notice the variety of things that compete for your attention.

Now in your mind’s eye, I would like you to look down, and way, way down below you’ll see a target, like an archery target. See that target clearly in your mind; see the coloured bands, see the bullseye; now in that bullseye is the pain you were experiencing. In a moment when you hear this sound, “swoosh”, you will be shot like an arrow, straight at that bullseye, when you hear that sound, “swoosh”, you will be shot like an arrow straight at the bullseye and go straight right through that bullseye into a place of no pain; as you pass through the bullseye, you will experience that pain intensify for a split-second; and then drift into a place where you are completely free of that. I want you to be brave about this.

So when you hear that sound you are going to be shot like an arrow straight through that target, so get ready.

Swoosh.

As you pass through it and out the other side. Into a place of no pain, calm open, awareness, just drifting in a place completely free of any unnecessary pain, drifting as a mind, free of all earthly ties, free of all distractions, as you drift there, in that wonderful feeling of open awareness, completely free of any unnecessary pain or discomfort.

I want you to now drift to the other side of the room that you are sitting in, drift to the other side of the room you're sitting in and from there see yourself sitting in that chair, you can see yourself sitting in that chair. I want you to notice that from the other side of the room as you watch yourself sitting in that chair, that every ounce of agitation and unnecessary pain has gone from your body and those thoughts have left your mind. You cannot experience them, you cannot feel any pain, you are completely separated from your body, and as you drift there, completely free, in a while you are going to drift back over to your body, but only as quickly as your body and mind can make all the changes, that will allow you that freedom, the freedom to experience that condition of freedom from any unnecessary pain , any time you wish to or need to, only then will you drift back over to your body and drop down into your body, free of agitation, free of distractions, take your time, as you are aware of the images and the sounds as you drift over, back into your body, unable to experience those old feelings. Go ahead, take your time. When you are fully reintegrated open your eyes and get on with your day taking that comfort with you.'

Breakdown of The Arrow Technique

The Symptom Scale

The Symptom Scale, or to give it its proper name the Subjective Units of Distress Scale (SUDS) is a scale of 0 to 10 for measuring the subjective intensity of distress currently experienced by an individual. The individual self assesses where they are on the scale. It can be used as a benchmark for the individual and for the practitioner to evaluate the progress of treatment.

In practice we do not provide a detailed scale with descriptions of what each point on it means. One of the benefits of asking a patient or client for a symptom scale score is that it is simple. Typically, before we get into delivering the technique, we simply ask the client, **“Close your eyes. Focus on that pain. On a scale of one to ten, where one is the best you can feel and ten is the worst, how does it feel right now?”**

We find it useful to use the symptom scale, especially when dealing with pain or fear for two reasons.

Firstly it gives the client a clear indication that progress has been made. If you haven't asked how bad it is at the beginning, then when they are free of pain at the end, they have nothing to compare it with. They may conclude, that maybe it just isn't painful in that moment, and will have to wait to see if the technique has made a difference. The comparison gives them a clearer insight.

The second reason we like the symptom scale, is because we can exploit a flaw in its use. This is that many people tend to overstate their experience of pain when first asked about it, perhaps because they wish to communicate clearly, that they are distressed by it. When then asked to engage in almost any procedure that it is suggested could help, and asked to symptom scale again; most will report their pain has decreased to some significant degree.

It seems that the way that the symptom scale question is asked can exacerbate this natural tendency to lean either way. This can help us frame the technique and create two emotional spikes in the session, which we can follow with some direct suggestions. The subject can more clearly see that progress has been made.

So immediately before you deliver the technique, and prior to the use of The Jacquin Fingerlock or The Jacquin Power Lift Induction, ask the subject to do something for you.

‘Close your eyes. Focus on that pain. On a scale of 1 to 10, one meaning it’s as good as it can be and ten meaning it’s very bad, how bad is that pain?’

As they pay attention to the severity of their pain, it creates a spike of emotion in them so when they open their eyes directly suggest a positive outcome regarding the session.

‘Open your eyes and look at me. When we have done this work, you will no longer be able to feel that. That will be fantastic will it not?’

When the body of The Arrow Technique is complete and the subject reorientates, they can be asked to rate severity again, in a way that gives them again an increased chance of not being able to experience the pain. This is likely to encourage positive and automatic self-suggestion.

Relaxation

People who are in a great deal of pain, may be in the habit of holding their physiology in a rigid way, breathing in a particular way, to avoid any unnecessary movement or impact, and this constant muscular tension, may increase their distress as well as hinder efforts to manage pain. Give them an opportunity to be safe enough, to let down their guard and give their muscles a chance to relax. So, before getting into the main body of the technique itself, it is useful to instruct, encourage and suggest relaxation. You will find with just a tiny bit of encouragement, many people in pain, can make an adjustment that will allow them to find a measured amount of pain relief, in addition to some mental or emotional release, and may strengthen their investment in the technique.

You can suggest deep relaxation. Understanding that deep relaxation does not to be that ‘deep’ to be pleasant, comforting, healing. When deeply relaxed you will not experience pain in the same way, you may feel none at all. The client can be encouraged to enjoy this safe space, to put their guard down and use this moment to quiet the mind, relax the body and release tensions.

‘As you sit comfortably, I want you for a moment to focus on your breathing. I want you to pretend and imagine that you are breathing in calmness and breathing out tension. And as you continue to breathe like this, just focus on your body and if you notice any tension anywhere just let it go as you exhale. Any time you need to move to make yourself more comfortable you can do so. In your mind softly and slowly repeat the word “Relax” four times. As you repeat that word you can begin to relax. Let every muscle relax. As you do that, you may become more aware of the ideas and images, that drift into the mind automatically. And allow yourself to drift down deeper and deeper into that feeling. Allow yourself to drift deeper and deeper, and imagine that is happening all by itself, as you listen to the sound of my voice.

Dissociation

In this section the participant is encouraged to imagine drifting out of their body, leaving it behind. This is an effortless way of distancing, detaching or disconnecting yourself from pain. The strain of pain is often decreased when imagined from a third person perspective. The person is already doing something differently. It is likely their experience will be different. This essentially involves thinking of the mind being separate from the body or rather the consciousness that you know yourself to be, being outside of your body. Often pain is weaker or non existent when we do this. It is as if your mind has the possibility to review this as a possibility and do that instead if it is OK to do so.

And now I would like you to allow yourself to have the experience of drifting out of your body, up out of your body; and drift away from your body and drift way, way up above your body, hundreds, thousands of feet above your body, leaving all physical ties behind, drifting way way up. Notice how it feels to be up there and as you look around notice the variety of things that compete for your attention.

Transformation

In this section the pain is symbolised as a target, way off in the distance. The centre is associated with being the most intense part of the pain. This transforms the specific qualities of the pain sensation. It is indirectly suggested that the pain is already in the past.

Now in your mind's eye, I would like you to look down, and way, way down below you'll see a target, like an archery target. See that target down there; see the coloured bands, see the bullseye, the centre of the target; now in that bullseye is the pain you were experiencing.

Emotion

An emotional spike is encouraged with the encouragement of anticipation at the thought of being fired toward the target, with a warning that the pain will briefly intensify. This focuses attention on the next moment. It is then suggested the participant will then go into a world of no pain. This opens the possibility of feeling no pain at all.

In a moment when you hear this sound, “swoosh”, You will be shot like an arrow, straight at the centre of the target, when you hear that sound, swoosh, you will be shot like an arrow straight at the bullseye until you go right through that bullseye into a place of no pain; as you pass through the bullseye, you will experience that pain intensify for a split-second; and then drift into a place where you are completely free of that. I want you to be brave about this.

So when you hear that sound you are going to be shot like an arrow straight through that target, so get ready.

Swoosh.

Distortion

The conscious aspect of the participant is associated with an open, spacious, emptiness, where there is no pain, no tension. It is suggested that there is an unnecessary portion of that pain. Pain that they don't need, suggesting that some or all of their pain will fall into this category.

As you pass through it and out the other side. Into a place of no pain, calm open, awareness, just drifting in a place completely free of any unnecessary pain, drifting as a mind, free of all earthly ties, free of all distractions, as you drift there, in that wonderful feeling of open awareness, completely free of any unnecessary pain or discomfort.

Double Dissociation

Another dissociation is encouraged. Closer to home this time, but still from a perspective where there is no pain. They are encouraged to see themselves, with no unnecessary pain or discomfort, from the other side of the room. As if they already know what that would look like.

I want you to now drift to the other side of the room that you are sitting in, drift to the other side of the room you're sitting in and from there see yourself sitting in that chair, you can see yourself sitting in that chair. I want you to notice that from the other side of the room as you watch yourself sitting in that chair, that every ounce of agitation and unnecessary pain has gone from your body and those thoughts have left your mind.

Direct Suggestion

As the participant holds this dissociated view of themselves, it is directly suggested that they can experience freedom from pain.

You cannot experience them, you cannot feel any pain, you are completely separated from your body.

Reassociation

Here the client is encouraged to integrate their sense of consciousness with their body again. This statement includes the binding language that they will reassociate in this way only as quickly as the necessary changes are made, changes that would give freedom from any unnecessary pain.

And as you drift there, completely free, in a while you are going to drift back over to your body, but only as quickly as your body and mind can make all the changes, that will allow you that freedom, the freedom to experience that condition of freedom from any unnecessary pain, any time you wish to or need to, only then will you drift back over to your body and drop down into your body, free of agitation, free of distractions, take your time, as you are aware of the images and the sounds as you drift over, back into your body, unable to experience those old feelings. Go ahead, take your time. When you are fully reintegrated open your eyes and get on with your day taking that comfort with you.

The Test Question

At the end of the session, ideally immediately after reassociation and return to normal alertness, symptom scale again, this time using your language to ensure they cannot access those feelings. The request is the same - self-assess and report to me how severe the pain is - however the way it is asked, makes the task and the experience quite different.

Open your eyes and look at me. Look at me. Where's the pain gone now? Try and feel it and find you cannot. Between 0 and 10, where has the pain gone now?

That's right. It will never bother you again.

Transcript - The Arrow for Emotional Pain Relief

‘As you sit comfortably, I want you for a moment to focus on your breathing. I want you to pretend and imagine that you are breathing in calmness and breathing out tension. And as you continue to breathe like this, just focus on your body and if you notice any tension anywhere just let it go as you exhale. In your mind softly and slowly repeat the word “Relax” four times. As you repeat that word you can begin to relax. Let every muscle relax. As you do that, you may become more aware of the ideas and images, that drift into the mind automatically. And allow yourself to drift down deeper and deeper into that feeling. Allow yourself to drift deeper and deeper, and imagine that is happening all by itself, as you listen to the sound of my voice.

And now I would like you to allow yourself to have the experience of drifting out of your body, up out of your body; and drift away from your body and drift way, way up above your body, hundreds, thousands of feet above your body, leaving all physical and emotional ties behind, drifting way way up. Notice how it feels to be up there and as you look around notice the variety of things that compete for your attention.

Now in your mind’s eye, I would like you to look down, and way, way down below you’ll see a target, like an archery target. See that target clearly in your mind; see the coloured bands, see the bullseye; now in that bullseye is the emotional pain you were experiencing. In a moment when you hear this sound, “swoosh”, you will be shot like an arrow, straight at that bullseye, when you hear that sound, “swoosh”, you will be shot like an arrow straight at the bullseye until you go right through that bullseye into a place of complete peace; as you pass through the bullseye, you will experience that emotional pain intensify for a split-second; and then drift into a place where you are completely free of that. I want you to be brave about this.

So when you hear that sound you are going to be shot like an arrow straight through that target, so get ready.

Swoosh.

As you pass through it and out the other side. Into a place of complete bliss, calm open, awareness, just drifting in a place completely free of any unnecessary hurt or emotional pain, drifting as a mind, free of all earthly ties, free of all distractions, as you drift there, in that wonderful feeling of open awareness, completely free of any unnecessary emotional pain or agitation.

I want you to now drift to the other side of the room that you are sitting in, drift to the other side of the room you're sitting in and from there see yourself sitting in that chair, you can see yourself sitting in that chair. I want you to notice that from the other side of the room as you watch yourself sitting in that chair, that every ounce of agitation and unnecessary emotional pain has gone from your body and those thoughts have left your mind. You cannot experience them, you cannot feel that any more, you are completely separated from your body, and as you drift there, completely free, in a while you are going to drift back over to your body, but only as quickly as your body and mind can make all the changes, that will allow you that freedom, the freedom to experience that condition of freedom from any unnecessary emotional agitation, any time you wish to or need to, only then will you drift back over to your body and drop down into your body, free of agitation, free of distractions, take your time, as you are aware of the images and the sounds as you drift over, back into your body, unable to experience those old feelings. Go ahead, take your time. When you are fully reintegrated open your eyes and get on with your day taking that calm, comfort and confidence with you.

Transcript - The Arrow for Increased Focus

‘As you sit comfortably, I want you for a moment to focus on your breathing. I want you to pretend and imagine that you are breathing in calmness and breathing out tension. And as you continue to breathe like this, just focus on your body and if you notice any tension anywhere just let it go as you exhale. In your mind softly and slowly repeat the word “Relax” four times. As you repeat that word you can begin to relax. Let every muscle relax. As you do that, you may become more aware of the ideas and images, that drift into the mind automatically. And allow yourself to drift down deeper and deeper into that feeling. Allow yourself to drift deeper and deeper, and imagine that is happening all by itself, as you listen to the sound of my voice.

And now I would like you to allow yourself to have the experience of drifting out of your body, up out of your body; and drift away from your body and drift way, way up above your body, hundreds, thousands of feet above your body, leaving all physical ties behind, drifting way way up. Notice how it feels to be up there and as you look around notice the variety of things that compete for your attention.

Now in your mind’s eye, I would like you to look down, and way, way down below you’ll see a target, like an archery target. See that target clearly in your mind; see the coloured bands, see the bullseye; now in that bullseye is the state of focus, you have been looking for. In a moment when you hear this sound, “swoosh”, you will be shot like an arrow, straight at that bullseye, when you hear that sound, “swoosh, you will be shot like an arrow straight at the bullseye and become more and more focussed on it until you go right through that bullseye into a state of total focus; as you pass through the bullseye, you will experience that sense of focus intensify; I want you to be brave about this.

So when you hear that sound you are going to be shot like an arrow straight through that target, so get ready.

Swoosh.

As you pass through it and out the other side. Into a place of total focus, calm open, awareness, just drifting in that world of total focus and calm, drifting as a mind, free of all earthly ties, free of all distractions, as you drift there, in that wonderful feeling of open awareness, total focus.

I want you to now drift to the other side of the room that you are sitting in, drift to the other side of the room you're sitting in and from there see yourself sitting in that chair, you can see yourself sitting in that chair. I want you to notice that from the other side of the room as you watch yourself sitting in that chair, that every ounce of agitation has gone from your body you are completely indifferent to any distractions, those feelings have gone from your body and those thoughts have left your mind. You cannot experience them, you cannot feel anything that could distract, interfere or agitate you, you are completely separated from your body, and as you drift there in that wonderful position of total focus, completely free of any distractions or agitation, In a while you are going to drift back over to your body, but only as quickly as your body and mind can make all the changes, that will allow you that freedom, the freedom to experience that condition of total focus, any time you wish to or need to, only then will you drift back over to your body and drop down into your body, free of agitation, free of distractions, take your time, as you are aware of the images and the sounds as you drift over, back into your body, totally focussed, unable to experience. Go ahead, take your time. When you are fully reintegrated open your eyes and get on with your day taking that focus with you.'

Back In The Room

After The Hypnotist has taken the subject into hypnosis and done their work it would be negligent if they did not then bring the subject out thoroughly, returning them to normal, of course while suggesting they can keep the changes. This is achieved with a wake-up procedure. Of course, the subject is not asleep, so it is odd to call it a wake-up. It is simply a way to be thorough and clear - the session has ended. If you have suggested hypnosis, then they may have ideas that they are in some kind of altered state; and if you do not do a wake-up, essentially suggest otherwise, the subject will emerge from hypnosis, eventually, but they might feel a little disorientated for a brief time.

The closing statements of The Arrow Technique act as a suitable wake-up procedure. Make them clear, and you will not need to do any of the following.

However if you wish to use the technique in a broader multi-faceted hypnotic intervention, or you want to make the completion of the technique totally clear, then the following ideas will allow you to.

The subject is taking your suggestions on board, so you can ensure they come out of hypnosis cleanly by suggesting that is so. It is a great opportunity to ensure that the subject return to normal awareness, feeling fantastic and ready to go in every way.

If the subject ends the session and still has their arm hanging in their air, then it is possible to link the lowering of that arm to their return to full alertness. In this approach no effort to count them out of hypnosis is required.

‘In a moment your arm will drift down only as quickly as you drift up, to full conscious alertness, completely free of that pain.’

Or you could get a little more elaborate. A good wake-up ratifies the work you have done; gives an opportunity for them to go back into a hypnosis, instantly if you wish to give them this ability; and ensures they are back to normal in every way:

‘In a moment I am going to count from one to five and on the count of “5” your eyes will open and you will be back at full conscious, wakeful alertness, everything back to normal in every way. It will be like you have woken up from a wonderful night’s sleep. You will understand hypnosis is not sleep; but the next time you get in bed and sleep, you will sleep wonderfully, better than you have for years and awake at an appropriate time, refreshed revitalised, feeling brand new.

One. Feeling less relaxed now, every muscle, nerve and fibre coming alive.

Two. A surge of energy is pouring in through your fingers, toes, up arms, legs, spine to the top of your head.

Three. Take a deep breath, fill your chest with energy-giving oxygen, it spreads to every muscle nerve and fibre.

Four. Take another breath as your head is being washed through with cool clear spring water, your entire body washed through, refreshed and revitalised.

Five. Eyes open, wide awake, back in the room.’

Or alternatively:

‘On the count of “five” you will be back at normal awareness, everything back to normal, feeling fantastic.

One. Feeling wonderful.

Two. To achieve your goals.

Three. With a feeling of freedom.

Four. Feel the force of that feeling.

Five. Eyes open feeling wonderfully alive.’

I will often clap my hands on the count of “Five” to add to the effect. This tends to startle the subject a little and helps put some distance between their condition when hypnotised and when not hypnotised. In this moment the doors to their hypnotic world close. It encourages some amnesia for the experience. You can further encourage this by immediately asking them a question that does not relate to the hypnotic experience they have just had. This could be related to a conversation you were having with them prior to the hypnosis, or just a trivial question. When they then try to recall the hypnotic experience they do so with the same ineffectiveness they might try to recall a dream that seemed vivid just moments earlier.

Health and Safety

The Arrow Technique as a stand-alone intervention does not require the use of hypnosis. Without that frame this is essentially a imaginative exercise that anyone can carry out safely. This is one of the reasons it has such utility. Anyone can learn it and apply it, in it's basic form.

What is paramount is that you ensure that if someone needs clinical attention, that they seek that attention, even in the absence of pain. Secondly it is important that you only remove unnecessary pain and discomfort. Finally ensure your suggestions have safeguards, that the solution will keep them safe, well and protected.

This video and booklet, is not intended to provide a full training in hypnosis or hypnotherapy. It is up to you to seek training and learn the skills that make one a competent hypnotist. However, as we often deliver the technique within this frame it is worth taking note of some guidelines to ensure health and safety is taken care of.

Health and Safety is almost completely in control of The Hypnotist. Being The Hypnotist brings responsibility with it. Be the best you can be and treat your subjects with the respect they deserve. Always demonstrate that you care and that they are in safe and competent hands. This makes the job easier anyway. Although hypnosis is unlikely to make an existing health condition worse it is wise to stick to sensible guidelines.

So, unless you are competently trained to do so:

Do not hypnotise people under 18 years of age. Do not hypnotise people who have a health conditions, including heart problems, breathing problems or psychiatric problems. Do not hypnotise people who are pregnant. Do not hypnotise people who are epileptic. Avoid people who are very drunk or high on drugs.

Be mindful of your participants condition. Ensure they are safe.

It is not that you cannot help these people. You most certainly can. It is unlikely hypnosis is going to carry an increased risk of problems for any of these people. It is just that you may need to pay special consideration to their condition. Good quality training helps with that.

Remember, you should not be treating anything with hypnosis, that you are not qualified to treat without hypnosis. So resist the temptation to frame what you do in medical or psychological terms beyond your qualified knowledge base.

Are you allowed to use words to help people let go of pain? We think this is your right. It is what we try to do when speak to our loved ones, when they are suffering. We are allowed to try to help others.

Do not take silly risks. Show the people you work with twice the amount of respect and attention you would in a normal situation. Ensure that your suggestions include remarks that they will not do anything that will hurt them or anyone else. Suggest that the changes can be made in a way that will keep them safe, well and protected. Finally give them the credit, reference their resources and abilities.

Abreactions and Other Surprises

Occasionally when a subject is hypnotised they have a negative emotional reaction that surprises them, and, if unprepared, The Hypnotist too. Sometimes this can be a large, unbridled reaction known as an abreaction. So be prepared. Fortunately an abreaction is uncommon, and if you do your job properly, extremely rare. The subject may be upset and cry, often with complete abandon. Sometimes they know why they are upset and sometimes they do not. It could be something you have said has acted as a trigger that has reminded them of something traumatic or a memory with which a negative emotion is associated. Sometimes it seems an abreaction is quite spontaneous—without any particular trigger. It is as if emotions we do our utmost to ignore, and outbursts we need to have but suppress, can lurk just beneath the surface of consciousness. Hypnosis might provide an opportunity for them to get out. Perhaps this has to do with the subject's expectations about hypnosis. An abreaction like this is nothing to be afraid of, although it is quite a shock the first time you see one. Managed appropriately you are not going to do anyone damage here and can ensure they come through it better off, if a little shaken.

Keep your suggestions clean. Do not regress people to places that could have been hurtful or harmful or frightening. Do not regress to their childhood. The worst thing The Hypnotist can do when faced with an abreaction is to alarm the subject even more by getting freaked out and looking anxious. Stay calm. If you are not going to start doing impromptu therapy then the best thing is to simply tell them that everything will be okay, that you will spend some time with them, and that in a moment you will bring them back up and they do not need to bring these memories and feelings back with them. Then do a proper wake-up, toning it down slightly from the usual suggestions to feel absolutely amazing. Everything back to normal will be okay. Spend some time with them afterwards to ensure they are okay.

Of course, for a hypnotherapist, an abreaction is an opportunity to deal with something. You have often found the splinter of a problem and can start to pull it out. But this is not a full training in therapy and does not assume you wish to use The Arrow Technique in that context. So if someone does appear to have an abreaction during a session of hypnosis, then resist the temptation to start doing therapy with them if that is not the place, but do use suggestion to suggest the reaction away.

An abreaction drill that you should learn, taught to me by the master of conversational hypnosis, Igor Ledochowski, follows. It begins with a repeated command to remain calm and this is followed by direct suggestions that the experience will change. Then the subject is grounded back into their physical reality with encouragement to focus on their surroundings

and finally they are reminded that they are safe. Repeat each line or the entire process as deemed necessary. Do not touch the person during the process as you do not want them to associate that touch with the abreaction.

‘Stay calm. Stay calm.

The scene is fading, becoming more and more distant, you can leave those feelings behind now.

You can feel the chair. Feel your feet on the floor.

And know you are safe.’

Follow this with a gentle wake-up. Welcome them back with a smile rather than a frown. Do not suggest they have issues to deal with and should seek therapy. They are likely to know why whatever it is that came up did come up and can make their own decisions.

On occasion a subject may get a little concerned as they feel they are going into hypnosis—if they associate this with losing control. If they have had panic attacks or a bad experience with drugs they may associate it with that. If they still wish to be hypnotised then it is important to let them feel in control and know that they can manage the depth they go to. This can be achieved by training them to take themselves in and out using the rehearsal element of The Jacquin Power Lift induction, associating arm raising with going into hypnosis and the arm lowering with returning to normal awareness.

Even more rarely someone may have a fit; perhaps they suffer from epilepsy or sleep apnoea. If they are not thrashing around when fitting, but have instead become very rigid, it can be difficult to spot immediately. Follow basic first aid advice, ensuring the person does not hurt themselves or choke as they fit.

Basic health and safety training is inexpensive. It will teach you how to save lives and much more in one day.

You should always ask people if they have any serious health conditions so you can make an

informed decision about whether to hypnotise them or not.

Using catalepsy with people who have arthritic conditions is obviously not advised. Think. Use common sense and a bit more. Look at the surrounding environment. Remove or steer clear of potential dangers. Be safe. Show that you care.

Morals and Ethics

It is often stated by hypnotists that you cannot make people do things they would not normally do, and certainly cannot make people do things that go against their morals and ethics. I absolutely disagree. Please keep in mind that morals and ethics are just a surface veneer maintained for social acceptance. When conscious critical thought is out of the equation it is possible and likely that moral and ethical boundaries are too. So in this sense The Hypnotist is responsible for the moral and ethical boundaries. The subject remains human and is capable of a full range of responses. Again I suggest you use common sense.

When hypnotising people to help them, you are not making people do things against their will; it is a collaborative effort to change their experience. Together you are bending their reality so that the subject can automatically experience the effect of these positive suggestion.

I repeat: you are responsible for the moral and ethical boundaries. Your participant is responsible for the positive changes.

You should always get permission to hypnotise. Even when your intention is to reduce suffering.

Final Thoughts

On all of our training courses and in our training products we emphasise that it is important to look beyond technique and let what you do become your art form. Practise, observe, stretch yourself—there is always more to learn. Then relax and do your work.

There is no need to settle for being average—be the best you can be. Of course, at first it is important to get your technique mastered, so work hard to do this. Once it is, be prepared to let your intuition guide you about how you apply this knowledge. Human beings are suggestible. Their personal reality, including their pain, is a fiction. Take that understanding and knowledge with you into every walk of life, whether you are speaking to your children, your colleagues or your friends. There is a vast amount of unnecessary pain and discomfort you can alleviate.

The words are not as important as the ideas. The techniques are not as important as your intention. Observe your subject; they will give you all the guidance you need.

Aim to leave your subjects and your audience with a new view of the world; give them a positive experience they will still recount twenty years from now.

We would love to hear from you regarding your use of The Arrow Technique and the results we know that you can achieve with it. Any words of testimonial are important to us, so let us know of your success by email or video. If you have questions or suggestions of how we can push this forward together, get in touch.

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